Scheduling New OB Visits including initial Telehealth

- OB Intake Telehealths will be done by a designated medical assistant, LPN, or RN assigned to the Happy Valley Prenatal Intake Resources (Schedule A, B, C, or D) using the OB INTAKE TELEHEALTH VISIT (defaults to 30 minutes)

- Prenatal Intake Video via MyHealth - *see step 2 for scheduling and scripting*

- For **early** Dating and Viability ultrasound follow up appts with the provider, these need to be a 30 minute Est Office Visit or 30 minute NP Office Visit **if they have not** had their OB Intake Telehealth visit yet

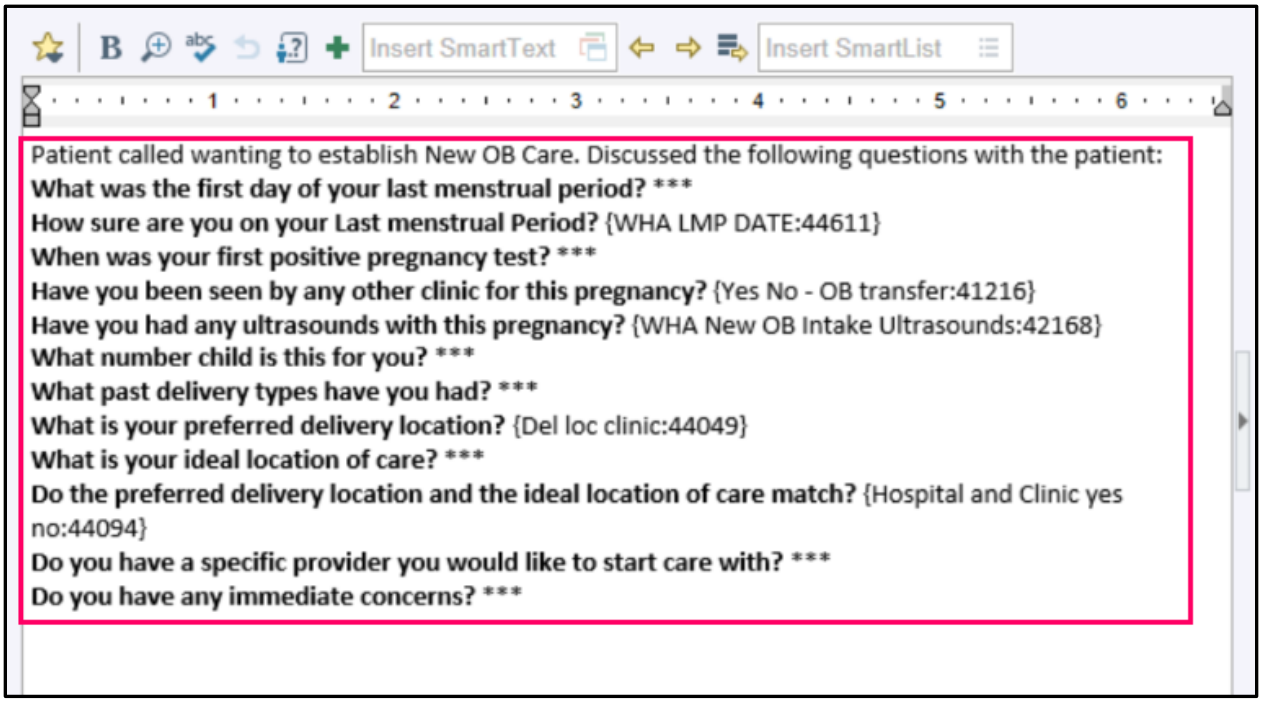
- **OB INTAKE EXTENED TELEHEALTH** block type will be used for patients needing interpreters or have technology/other barriers to accessing Myhealth (defaults to 75 minutes) – **use Manual Search!**

- **All OB patients** will need to have an OB Intake Telehealth visit initially (this will be on the Happy Valley Template)

- **ER Follow up NOBs –**
  - Update patient registration, or register completely if patient is new. Registration update must be completed before moving on to the next step
  - **Complete OB intake telephone encounter** ensuring you document in the "**Do you have any immediate concerns**" section – "yes, seen in _____ER on _____date, patient connected with advice team"
  - The scheduler should **not** schedule the patient for an OB Intake Telehealth
  - Route encounter to OB coordinator pool
  - Warm transfer New and Established patients to Live Advice

Step 1: OB Intake Telephone Encounter

- Scheduler creates OB Intake Telephone Encounter using **.WHANOBINTAKE** and routes to site's OB Coordinator Pool
    - Use .WHANOBINTAKE dot phrase:

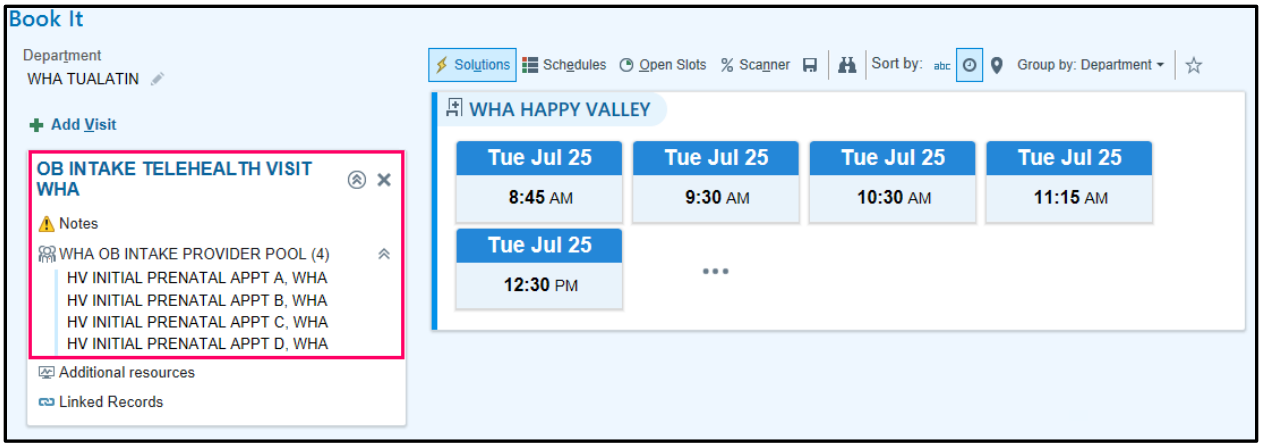


- If patient indicates that they are unsure of next steps, offer scheduling routine prenatal care or options counseling.
    - *Scripting: Would you like to schedule routine prenatal care?* Pause…*or a visit to discuss your pregnancy options?*
    - If options counseling desired see options counseling workflow

Step 2: Schedule OB Intake Telehealth

For scheduling, utilize appropriate visit type and auto search for questionnaire to prompt scheduling.

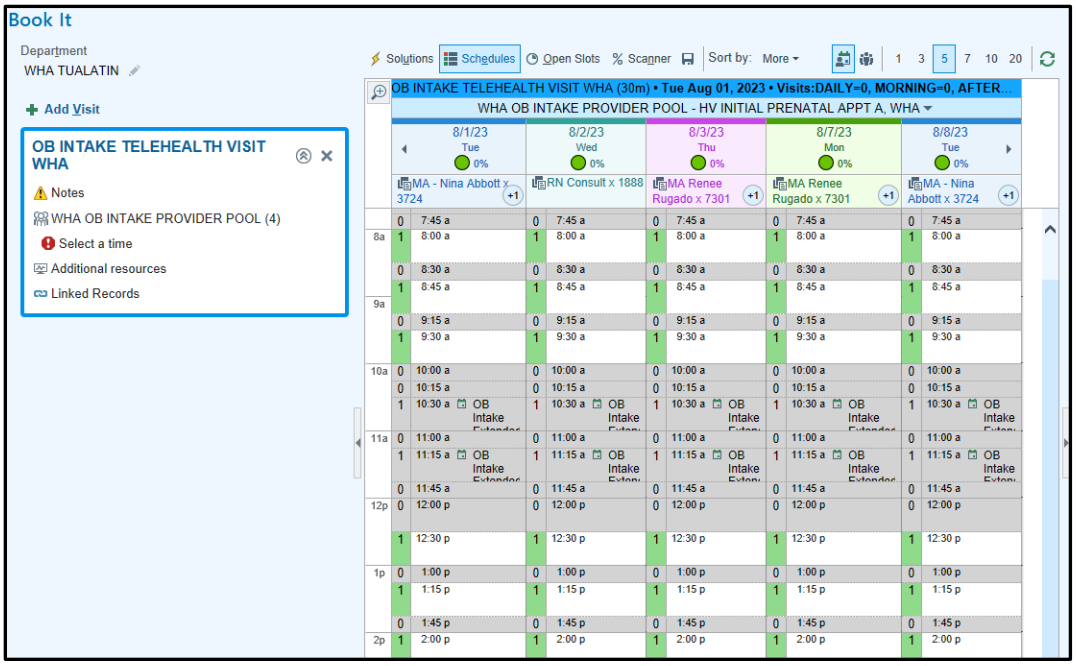
- When scheduling any appointment using the **OB INTAKE TELEHEALTH** visit type: Utilize this scripting: "*To start your care we will schedule you for an intake with a clinical team member. We will be sending you a link via your MyHealth account – please make sure to activate and check your account prior to your appointment. This video is 30 minutes please make sure to watch it before your first scheduled appointment.*"



- **OB INTAKE TELEHEALTH** visit type is defaulted to 30 minutes. It will automatically pull in only the Happy Valley Prenatal Intake Resources

For patients who need an interpreter or have technology limitations and/or are not able to access MyHealth, manually search for the new block type **OB INTAKE EXTENED TELEHEALTH on the Happy Valley Template.** There will be one **OB INTAKE EXTENED TELEHEALTH** appointment per day, per Resource (usually at 10:30am or 3:00pm) & it will thaw off 48 hours prior if not filled. Use BOTH

OB INTAKE EXT TELEHEALTH spots – creating a 75 minute long appointment for these patients.



Schedule visits top down, offering the first available appointment in the morning or after lunch, to ensure patients are back to back with no open space between them

Step 3 – Schedule 8 week Telehealth / OB Primary Consult Telehealth

Ideally this Telehealth visit would be scheduled with the same provider that will be scheduled to do their OB Workup. The 2nd option is a provider within the preferred provider's call group.

If patient is declining this visit please utilize the following scripting:

- *"In order to proceed with your 12 week ultrasound and in person provider visit, we have to ensure that you complete this appointment. This appointment is necessary for your provider to evaluate your pregnancy and ensure we have the correct ultrasound order in place for that 12 week ultrasound and in person provider visit."*

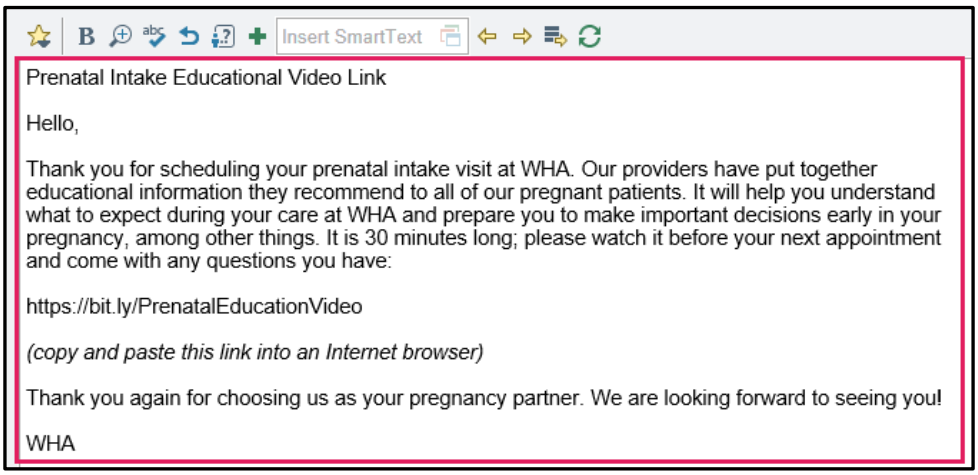
Step 4 – Schedule D&V US and OB Workup

The Dating and Viability ultrasound/ OB Workup / lab should be scheduled between

11.4-13.6 weeks -- ***ideally prior to 13.4 to allow time for repeat NT if needed***

- **Remind patient to activate/check MyHealth –** messages are sent the following business day for all appointments that are not next business day
- If you schedule a patient for their OB Intake Telehealth (on the Prenatal Happy Valley Resource) within 24 business hours please send the patient a MyHealth message using the dot phrase

  - PI video smart phrase is: **.WHAPIVDEO**
  - Check mark: "Do not allow patient reply"
  - Click send (bottom right corner)



- If a patient comes to the OB Intake Telehealth appointment without watching the video the MA/LPN will still complete the appointment. They will additionally encourage the patient to watch the video prior to their OB Primary Consult Telehealth appointment.

Step 5 – Care Teams

Enter the Care Team provider in the patient's registration:

The patient's Care Team provider needs to be the provider the patient and the scheduler identify as the patient's preferred provider for pregnancy (do **not** default to the provider the OB Workup is scheduled with)

Two WHA Care Team providers need to be entered at this time:

1. The patient's preferred provider for this pregnancy
    a. Relationship listed as **Obstetrician**
    b. Start date of date patient informed us they are pregnant
    c. End date is patient's rough 3 week postpartum date
2. The patient's preferred provider for GYN care:
    a. Relationship listed as **Gynecologist**
    b. Start date is 1 day after their listed "3 week postpartum" date
    c. No end date

If there are any past WHA providers already listed as Gynecologist or Obstetrician, expire these as to keep the patient's Care Team as accurate as possible

When scheduling all future OB visits, schedulers should be reviewing the Care Team with the patient to ensure it is accurate. If a patient or scheduler identifies that the Care Team entered is no longer who the patient is preferring to see for their pregnancy, the scheduler will update the listed Care Team members and send a Staff Message to the Care Team provider's Pod to update the pregnancy episode of care if indicated

FAQs:

- All patients should be strongly encouraged to activate and login to their MyHealth and shared the importance of watching the video prior to their MA/LPN OB Intake Telehealth on the Prenatal Resource on the Happy Valley Template
- Each HV Prenatal Intake Resource will have one 75 minute spot a day that is for patients who need an interpreter or have technology limitations and cannot watch the video prior. Use the **OB INTAKE EXTENED TELEHEALTH** block in the Schedule screen
- Each morning messages are sent to the patient who called the prior business day and scheduled an OB Intake Telehealth (no matter how far out the visit is)
    - If you schedule a patient for their OB Intake Telehealth (on HV or Nurse rooming) within 24 business hours, Send the patient a MyHealth Patient Message using the dot phrase *(see step 4 for more info)*
- If a patient comes to the OB Intake Telehealth appointment without watching the video the MA/LPN will still complete the appointment. They will additionally encourage the patient to watch the video prior to their OB Primary Consult Telehealth appointment.

Q: What if a patient calls with an unknown LMP or 10-14 week gestational age?

Answer:

- Schedule the patient for an OB Intake Telehealth – encouraging the patient to sign up for MyHealth if they haven't done so already and watch the Prenatal Intake video prior to their OB Primary Consult Telehealth appointment
- Schedule patient for OB Primary Consult Telehealth appointment
- Schedule patient for their OB Workup, 50 minute Ultrasound, and lab prior to the 13.6 week mark

Q: What if a patient who calls is at a 14.0 weeks – 19.6 weeks gestational age?

Answer:

- Schedule the patient for an OB Intake Telehealth on the Prenatal resource on the HV Template – encouraging the patient to sign up for MyHealth if they haven't done so already and watch the PI video prior to their OB Primary Consult Telehealth appointment
- Schedule patient for OB Primary Consult Telehealth appointment
- Patient to be scheduled for 60 minute ultrasound and lab within 1 week of their OB Primary Consult Telehealth visit

  Note: The ultrasound is 60 minutes to accommodate a potential early anatomy scan, if patient is 17-19 weeks gestational age we should schedule the patient at the 20-21 week gestational age mark, this will decrease the need for a repeat ultrasound

Q: What if a patient who calls is at 20+ week's gestational age?

Answer:

- The receptionist will schedule the patient for the OB Intake Telehealth visit
- The receptionist will determine if an Anatomy Scan is needed based on patient's response to the date of their last Ultrasound (see pregnancy wheel tip here for quick reference). If Anatomy scan is needed, the receptionist will schedule an 60 minute ultrasound and an **in person OB WORKUP (30 minutes)** visit to follow
- If an ultrasound is not needed patient is to be scheduled for an in person provider visit for their **OB WORKUP (30 minutes)**

Q: If a patient calls with a genetic testing decision?

Answer:

1. PS team to look for telephone encounter labelled **Other - Genetic testing decision follow up**

a. If telephone encounter present they will document patient decision in this telephone encounter and route to the site OB Coordinator pool

If no telephone encounter labelled **Other - Genetic testing decision follow up.** PS team member to look for **OB Intake telephone encounter** and will document patient decision in this telephone encounter and route to the site OB Coordinator pool

OB Coordinator Role within NOB visits

1. Reviews Telephone Encounter and scheduled visits to ensure accurate scheduling

- OB Coordinator to review NOB Intake telephone encounter question 4 regarding **records, past care, and ultrasound**
- After review, the OB Coordinator will enter into the Intake encounter the information below and then defer out to day after the patient's scheduled OB Intake Telehealth visit. Add:
  - OB Intake Telehealth date –
  - 8 week Telehealth date –
  - OB Workup date -
- If patient is 20+ weeks gestation and ultrasound is scheduled, send an **urgent staff message** to clinician that will be seeing the patient for their OB Workup requesting ultrasound order
  - Postpone telephone encounter for 3 business days and check back to ensure correct order entered and link to visit
  - If the order **has not** been entered after 3 business days, resend **urgent Staff Message** to clinician and **copy** the Patient Care Supervisor/Practice Supervisor for the site. Note in that Staff Message: "2nd request, postpone 3 business days again and follow up and link order"

- **OB Transfer** patients that No Show the OB Intake Telehealth, that we cannot reschedule them for that visit prior to their US and Workup, keep the visits as is and Staff Message the provider:
  - "For awareness, the OB Intake Telehealth was not done due to scheduling and GA conflicts" – Also add if an US order is still needed.

2. Postpone telephone encounter to day after Intake to ensure the visit was completed

- If the Intake **was not** completed, call the patient 3 times prior to 8 week telehealth to attempt to reschedule the intake
- If the Intake **was completed** ensure 8 week and 12 week visits are still scheduled

3. Postpone Telephone Encounter to after 8 week visit

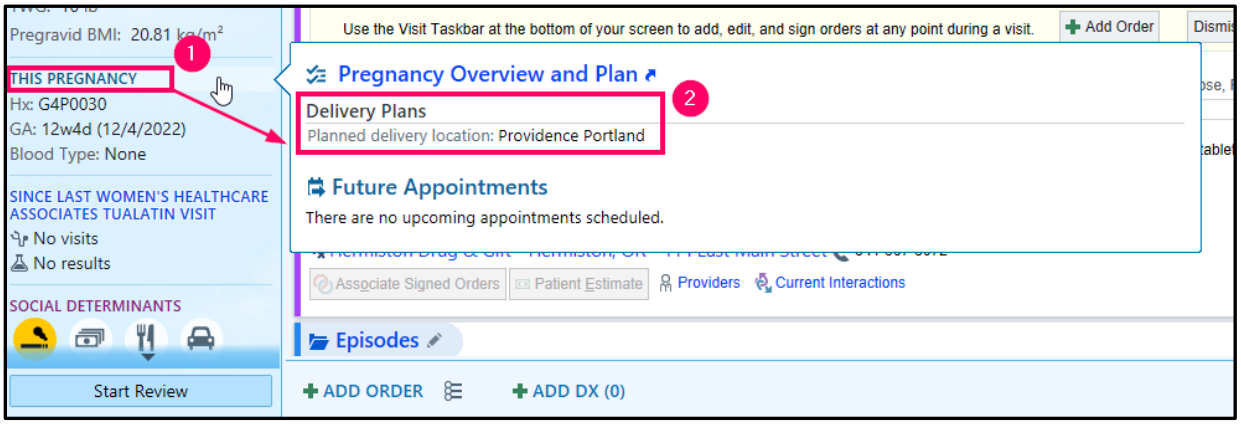
- If patient no showed Intake and 8 week telehealth visit, call to attempt to reschedule x1
- If unable to reschedule both visits cancel the 12 week ultrasound and OB Workup
- If patient no showed 8 week visit only
  Make 3 calls to attempt to reschedule this visit prior to the 12 week visit

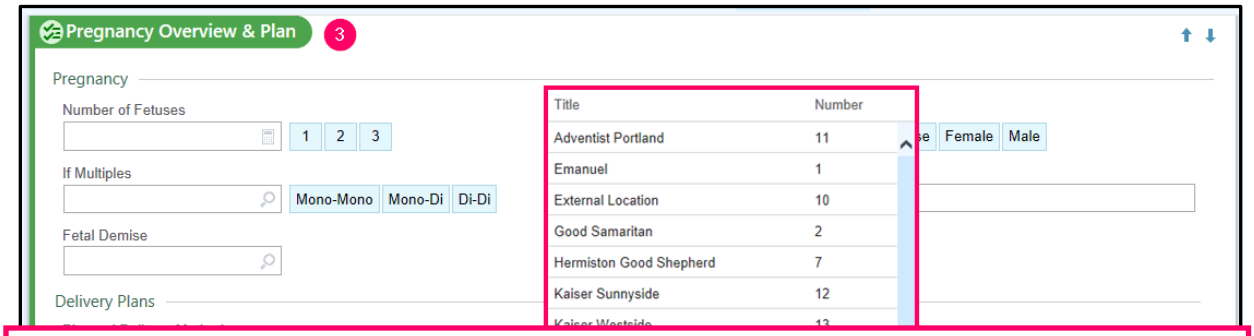
If patient has completed 8 week telehealth:

- Review assessment & plan in the pregnancy problem in the chart to check ultrasound order
  - In chart on snap shot click Pregnancy
  - In pop up screen review top section problem details
- If patient desires NIPT or declines genetic testing. This will be indicated by the ultrasound order, which should be for Dating and Viability only
- If patient desires SEQ testing, this will be indicated by SEQ Ultrasound/D&V US with NT order

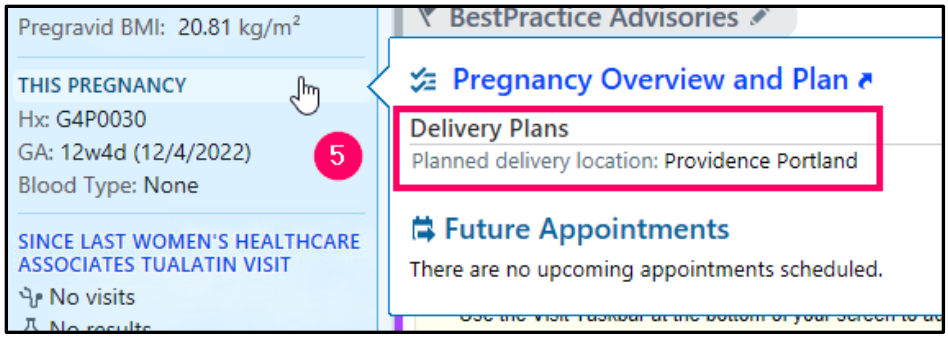
- If there is not an Ultrasound order entered yet, the OB Coordinator Staff Messages the Provider that did the 8 week visit to enter an US order and follows up with that Provider until one is entered
- **Checking and finding the planned delivery location**

(see steps below)





1. Under patient Storyboard in the Chart, hover over **This Pregnancy** to expand the Pregnancy Overview and Plan
2. If the delivery location has been entered it will appear under **Delivery Plans**
3. If no delivery location is listed but you find the information documented elsewhere in the chart, click on **This Pregnancy** to open the overview
4. Go to **Planned Delivery Location** and select the delivery location from the list



After entering the location, continue any work being done in the OB Intake telephone encounter or documentation. There is no need to sign the encounter as navigating away from the Pregnancy Overview and Plan section will automatically save the location entered

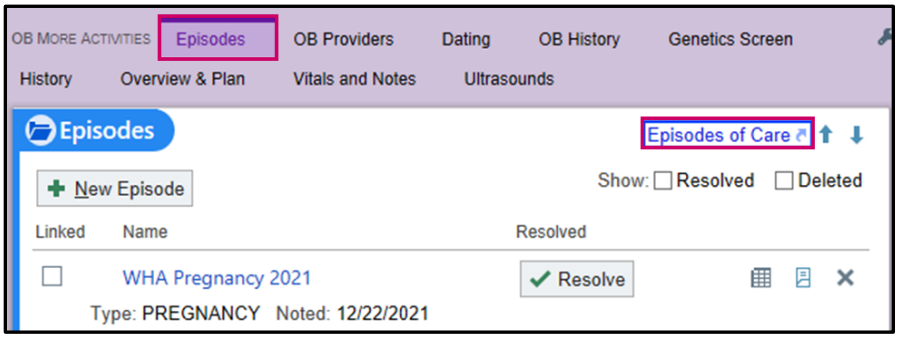
5. Hover over **This Pregnancy** again to check if the location was added correctly

Note: If you see "**No pregnancy episode available**" do these steps below:

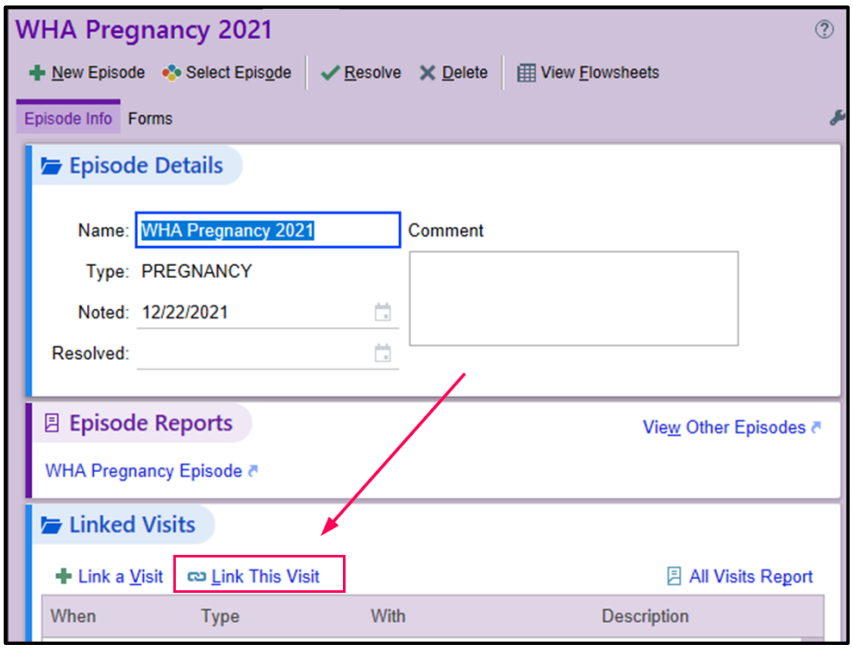
If you see "**No pregnancy episode available**." click Close. This occurs because the Telephone Encounter wasn't linked to the episode when the episode was created.



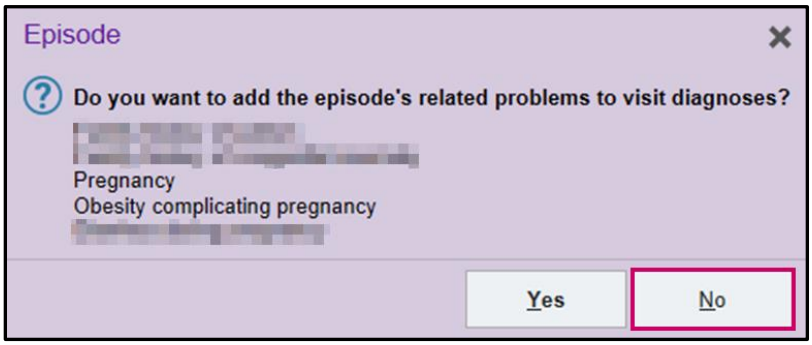
To fix that, go to the **Episodes** activity and click the hyperlink **Episodes of Care**



Click **Link This Visit**



If you get a pop up asking to add related problems to visit diagnosis **click NO**



There will be a slight delay after clicking back into the pregnancy overview and plan, but when it loads you will be able to add the patients delivery location. From here you can do the rest of your workflow

Note: You will have to do this for most patients because of the nature of the workflow. Once the encounter gets linked to the episode you **won't** need to link it again.

4. Go to patient's Appointment Desk and look at the Active Requests tab to ensure order matches the provider assessment

- Click on Active Request Tab
- Double click on the order
- Review First trimester Exam Requested section
- If order matches link order to ultrasound visit
- If order **does not** match text in Pregnancy problem, send an urgent staff message to clinician that performed 8 week visit to enter correct ultrasound order
    - Postpone telephone encounter for 3 business days and check back to ensure correct order entered and link to visit
    - If the order has not been corrected or entered after 3 business days, resend to clinician and copy the PCS/PS for the site, note in the staff message 2nd request, postpone 3 business days again and follow up and link order

If a **patient is undecided** on genetic testing after the 8 week visit or if the OB Coordinator is unsure of which testing the patient desires as it is not documented and patient's US and OB WorkUp are more than 5 days out please follow the following steps:

- OB Coordinator will document in the OB tracking telephone encounter –
    a. Patient undecided on genetic testing, will follow up with patient in three days *Postponing the telephone encounter for three days to allow for patient to have time to decide, call insurance, etc.
- Once three days have passed the OB Coordinator will create a new telephone encounter for this documentation
    b. Reason for Call: **Other** and reason notes should say **Genetic testing decision follow up**
    c. OB Coordinator only needs to make one call attempt
    d. OB Coordinator will call the patient to determine if patient has made a decision around genetic testing. Please use the following scripting:

e. Initial Scripting:

I'm calling to see if you've made a decision on whether you plan to schedule optional genetic screening or if you are choosing to decline. Your options were discussed at your prenatal intake appointment and include:

- Sequential Screen **OR** QNatal / NIPT *(not both)*
- Carrier screening for Cystic Fibrosis (CF) and/or Spinal Muscular Atrophy (SMA) ***This can be in addition to Sequential Screening or NIPT screening or they can perform these tests without the Sequential or NIPT screening*
- Opting out

f. If patient has billing questions:

- These are optional screens and are not required for routine care. It is possible that your insurance company will not cover any of these so it is important that you verify your benefits prior to deciding. If you have any additional questions around cost I can connect you with one of our financial counselors to discuss further

g. If patient has medical questions:

- I will need to send your question to one of our medical assistants to review your question. I will send them a telephone encounter with your question and they will give you a call back sometime today or tomorrow to discuss further

h. If you are leaving a voicemail:

- *I'm calling to remind you to call us back with your decision regarding whether you plan to schedule optional genetic*

screening or if you are choosing to decline.

- *If you have not reviewed the materials provided at your intake appointment, please do so, then call your insurance company for coverage information, and call us with your decision as soon as possible at least 5 business days prior to the day of your appointment on xx/xx/xx (date of OB Workup) as we need to ensure we have the right order in place for your visit. If we do not hear back from you, we will assume you wish to opt out.*

i. Once you have the patient's decision or if it is 3 business days prior to the visit please follow *step 3* of this workflow to ensure correct orders are placed and linked to the patient's upcoming ultrasound

- If any orders need to be cancelled after the correct order is placed, Staff Message the POD 1 MA pool at the appropriate site to have the MA cancel the incorrect order

5. Postpone to after the 12 week visit to ensure patient had visit or reschedule if needed

6. Once 12 week visit is completed, sign the OB Intake encounter

Note: If the patient miscarries during the first trimester, OB Coordinator will send a staff message to WHA Billing & Coding Pool indicating end of pregnancy. OB Coordinator will document in the telephone encounter that a staff message was sent to the WHA Billing & Coding Pool then sign off the telephone encounter

Financial Coordinator Role within NOB visits

1. Runs an EPIC Workqueue indicating patient is scheduled for NOB visits to obtain benefits
2. Verifies benefits and contacts patient with insurance coverage
3. Financial Coordinator enters deposit on patient's account for collection at the appropriate gestational age